Adjuvant dabrafenib and trametinib after removal of BRAF-mutated stage III melanoma

Your diagnosis and treatment setting NCCP 00415b: adjuvant treatment after complete resection of stage III melanoma with a BRAF V600 mutation. This route is distinct from treatment of unresectable or metastatic melanoma.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Adjuvant oral treatment

Active medicines: Dabrafenib + Trametinib

Emetogenic risk: Moderate to high (oral)

Medicine	Dose / route	When
Individual anti-sickness / rescue plan	To be completed by the prescribing team if needed	As specified in the individual plan; no routine schedule issued here Source emetogenic category and local policy guide the decision. No medicine, dose or administration order is selected by this draft.

Additional phase-specific support

- Fever ≥38°C: hold both medicines and contact the team; restarting requires the agreed plan after at least 24 hours symptom-free. Recurrent episodes need their own assessment. Dabrafenib is moderate-to-high emetic risk and trametinib minimal-to-low; the combined plan must reflect both.
- Discuss fertility and pregnancy precautions before starting. Use an effective alternative to hormonal contraception during treatment and for two weeks after dabrafenib and 16 weeks after trametinib; dabrafenib can make hormonal contraception less effective. The team must confirm the method and the later applicable end date for the combination. Do not breastfeed during treatment; ask when it is safe to resume. Report a possible pregnancy promptly.
- Fever is a particular concern. At 38°C or above, withhold both medicines and contact the oncology team promptly under the written fever plan; recurrent episodes may require action at the first symptoms. Infection and dehydration also need assessment. Restart only when the team tells you. Urgent help is also needed for vision change, severe rash, chest pain, breathlessness, bleeding or severe muscle symptoms.
- This is not an issued prescription. The clinician and pharmacist must complete and authorise any supportive medicines.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00415 v5 · p6–7 · NCCP regimen and local supportive-care policy

Sources checked 2026-11-06 · SACTCheck v0.84.40 · Regimen-level drafts; prescriber and oncology-pharmacy verification required